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What to Expect After a Concussion

A guide for you and your family — written by your clinician

This handout is for patients, parents, and support people. It explains what a concussion is, what recovery looks like, and what to do in the next few weeks. Share it with the people around you — partners, teachers, employers, coaches — so everyone has the same information.

What a concussion is

A concussion is a brain injury. It happens when a bump, jolt, or rapid movement of your head or body causes your brain to move inside your skull. You don't need to have been knocked out or hit in the head directly to have had a concussion — rotational and whiplash forces can cause one too.

A concussion is a **functional** injury, not a structural one. That means scans (CT, MRI) usually look normal. The problem is in how the brain is working, not in physical damage you can see on an image. This is why your clinician is using symptom-based assessments rather than sending you for a scan.

Common symptoms

You may experience one or many of the following. It's common to have some symptoms but not others, and for symptoms to change day by day.

Physical: headache, dizziness, balance problems, nausea, fatigue, sensitivity to light or sound, blurred vision, neck pain.

Thinking: difficulty concentrating, memory problems, feeling “in a fog,” slowed thinking.

Mood: irritability, sadness, anxiety, emotional ups and downs.

Sleep: trouble falling asleep, trouble staying asleep, sleeping more than usual, feeling unrefreshed on waking.

How long does recovery take?

- **Adults:** typically 10 to 14 days.
- **Adolescents:** typically up to 4 weeks.
- **A smaller group (about 10–20%)** have symptoms lasting longer. Your clinician will reassess if recovery is slower than expected and adjust the plan.

Recovery is rarely a straight line. You may feel better, have a setback, then feel better again. A bad day does not mean you are going backwards.

What to do in the first 48 hours

- **Relative rest**, not total rest. Reduce physical and mental load. Light activities of daily living are fine.
- **Reduce screen time** — phone, TV, computer. Not zero, but less.
- **Sleep**. Prioritise it. Regular bedtime and wake time.
- **No alcohol, recreational drugs, or contact sport.**
- **Hydration and regular meals.**
- **Paracetamol** is fine for headache if needed. Avoid ibuprofen and other anti-inflammatories in the first 48 hours.

What to do from day 2 or 3 onwards

Start moving. Light activity — a walk, a gentle bike ride — is good for recovery, even if it brings on mild symptoms.

The rule is **sub-symptom threshold**: if an activity makes your symptoms clearly worse, dial it back a bit. If it doesn't, keep going. You don't need to avoid everything that causes any symptom at all. Your clinician will set the specific plan.

This is a big change from older advice to “rest in a dark room.” The current evidence is clear that gentle activity from day 2–3 actually helps you recover faster.

Going back to school, work, and sport

Your clinician will give you a staged plan. The principles are the same whatever you're returning to:

1. **Light activity at home**
2. **Light activity away from home** (e.g. desk work from home)
3. **Partial return** with reduced load
4. **Full return** with modifications
5. **Full return**, no modifications

For sport there's an extra stage — non-contact training, then full contact, then game play. Stay at each stage for at least 24 hours before moving on. If symptoms come back, drop back a stage.

When to seek urgent medical help

Go to the emergency department or call an ambulance if you develop any of the following:

- Worsening severe headache that doesn't improve
- Repeated vomiting
- Becoming drowsy, difficult to wake, or losing consciousness
- Seizures
- Weakness, numbness, or loss of coordination
- Slurred speech
- Confusion that worsens, or unusual behaviour
- Fluid or blood leaking from the nose or ears

Things that help recovery

- **Sleep**. 8+ hours for adults, 9–10 for adolescents.

- **Hydration and balanced meals.**
- **Light exercise** from day 2–3, built up gradually.
- **Connection.** Talking to friends, family, and your clinician. Isolation prolongs recovery.
- **Asking for modifications** at work, school, or home. People want to help — tell them what helps.

Things that slow recovery

- Trying to push through and ignore symptoms
- Total isolation and extended bed rest
- Alcohol, cannabis, recreational drugs
- Returning to sport too early
- Not telling anyone about your symptoms

If recovery is slow

If you still have significant symptoms at 4 weeks, that's when your clinician will reassess and may add targeted treatment — vestibular rehabilitation for dizziness, manual therapy for neck-related symptoms, structured exercise programmes for exercise intolerance, psychological support for mood or anxiety.

Slow recovery is not the same as permanent injury. Most people with persistent symptoms do fully recover. It just takes longer and needs a more targeted plan.

Questions to ask your clinician

- What are my specific red flags?
- What activities should I do today? This week? Next week?
- When am I cleared to drive?
- When am I cleared to return to work or school?
- When am I cleared to return to sport — non-contact, then full contact?
- What's the plan if symptoms get worse?
- When is my next review?

What your clinician has written for you today

Date of injury:

Date of this review:

Next review scheduled:

Specific advice for this week:

Specific red flags to watch for:

Clinician name and contact: